



EDU-FOOT

Step Ahead of Amputation

EDU-FOOT ESSENTIAL

Diabetic Foot Screening & Early Intervention

Every Nurse a Limb Saver

1. COURSE PHILOSOPHY & RATIONALE

The South African Diabetic Foot Crisis

Diabetes mellitus affects approximately **12.7% of South African adults**, with Gauteng recording the highest provincial prevalence (**15.2%**) (International Diabetes Federation, 2021). At Leratong Hospital, **96% of patients admitted with diabetic foot sepsis proceed to amputation**, and 70% of these are **major amputations** (Turner et al., 2024). Nationally, DFUs precede **84% of all diabetes-related amputations** (Njokweni, 2024).

The economic burden is profound — a single major amputation costs the public sector over **R500,000** in acute and rehabilitation care, excluding lost productivity.

Evidence for Screening

Structured diabetic foot screening — particularly using the **10 g monofilament test** — reduces ulcer incidence by **45–85%** (Singh et al., 2005; Bakker et al., 2016) and has **90% sensitivity** for predicting ulceration (Adler et al., 1997). However, only **12% of Gauteng PHC facilities possess monofilaments**, leading to an estimated **73% of neuropathy cases being missed** (Njokweni, 2024).

Framework Alignment

This course aligns with:

- **Amputation Prevention Protocol (APP)** – Leratong Hospital model (Njokweni, 2024)
- **IWGDF Guidelines** (Schaper et al., 2016) – global standards for DFU care
- **Gauteng Limb Salvage Programme** – provincial amputation reduction strategy
- **ICDM Framework** (NDoH, 2014) – integrated chronic disease care

Philosophy

Core Value	Description
Simplicity Over Complexity	Teach practical, evidence-based steps usable at clinic level
Competency Over Completion	Certification based on skill mastery, not just attendance
Contextual Adaptation	South African realities, rural–urban relevance, multilingual
Sustainability	Integrates into existing PHC workflows without disruption

“Every nurse trained becomes a sentinel against preventable amputation, a guardian of mobility, and a beacon of hope for diabetic patients.” — Mazizi Njokweni

2. COURSE STRUCTURE (2-DAY OVERVIEW)

Day	Sessions	Core Competencies	Learning Methods
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Day 1: Detection & Risk Stratification	1. The South African Crisis 2. The 5-Minute Foot Screen 3. Risk Categories & Actions	Perform systematic foot screening, apply 10 g monofilament, classify risk	Lecture, case video, 4-station rotation
Day 2: Basic Care & Referral Pathways	4. Foot Problems You Can Manage 5. Red Flags & Referral Pathways 6. Clinic Integration & Action Planning	Manage minor foot issues, use referral tools, develop local workflows	Demonstration, role play, workshop

Format: 70% Practical · 30% Theory

Duration: 14 Contact Hours (2 Days)

Certification: EDU-FOOT Foot Screening Competency Certificate (1-Year Validity)

CPD Points: 10 CEUs (Pending SANC Accreditation)

3. LEARNING OUTCOMES

Clinical Competencies

Graduates will be able to:

1. Perform **comprehensive foot screening in ≤7 minutes** using the ASK–LOOK–FEEL–TEST–ACT protocol.
2. Correctly apply the **10 g monofilament to three standardized sites**.
3. Palpate **dorsalis pedis (DP) and posterior tibial (PT) pulses** accurately.
4. Classify risk using the **Green–Yellow–Orange–Red framework**.
5. Recognize **emergency presentations** and initiate urgent referrals.
6. Provide evidence-based **first-line care for dry skin, callus, and nails**.
7. Navigate the **APP algorithm** and use **tele-referral via WhatsApp**.
8. Document findings using the **EDU-FOOT Screening Form**.

Professional Development

- **80% of participants** report improved screening confidence.
- **90% implement screening** within one month post-course.
- **100% of emergencies referred** within 24 hours (target indicator).

4. CORE MODULE CONTENT

DAY 1: DETECTION & RISK STRATIFICATION

Session 1: The South African Crisis

- **Case video:** “Thabo’s Journey” – ulcer to amputation (Leratong Hospital, 2023).
- **Review of 2024 Leratong APP audit** (Njokweni, 2024).
- **Discussion:** “What could have been prevented?”
- **Activity:** “One Change I Commit To” card.

Session 2: The 5-Minute Foot Screen

ASK–LOOK–FEEL–TEST–ACT sequence:

- **ASK:** symptoms, footwear, ulcer history.
- **LOOK:** skin, nails, deformities, red flags (blood-stained callus = 80% ulcer predictor).
- **FEEL:** DP/PT pulses, temperature, refill.
- **TEST:** 10 g monofilament or Ipswich Touch Test (Rayman et al., 2011).
- **ACT:** Document + classify.

Hands-On Stations:

1. **LOOK & IDENTIFY** – common findings.
2. **FEEL PULSES** – DP and PT practice.
3. **TEST SENSATION** – monofilament technique.
4. **DOCUMENTATION** – EDU-FOOT Screening Form.

Session 3: Risk Categories & Immediate Action

- **Green (Low), Yellow (Moderate), Orange (High), Red (Emergency)** (Schaper et al., 2016).
- **Case exercises:**
 - “Mama Zinhle” (High Risk).
 - “Baba John” (Emergency).
 - “Sister Thandi” (Moderate).
- **Day 1 mini-OSCE** and peer feedback.

DAY 2: BASIC CARE & REFERRAL PATHWAYS

Session 4: Foot Problems You Can Manage

- **Dry skin** → urea-based moisturizer (Baker, 2020).
- **Callus** → never cut; refer for podiatry debridement (Young et al., 1992).
- **Nail care** → cut straight, avoid sides; refer if ingrown or fungal.
- **Wound first aid** → saline only, no antiseptics, non-adherent dressings.
- **Practice:** Application on models.

Session 5: Red Flags & Referral Pathways

- **APP Algorithm:**
 - *Hot Foot* → Infection dominant → General/Orthopaedic Surgery.

- *Cold Foot* → Ischaemia dominant → Vascular Surgery.
- *Mixed* → Both urgent referrals.
- **Tele-referral:** WhatsApp Template.
- **Photo skills:** lighting, scale, background consistency.
- **Role-play:** Urgent referral scenario.

Session 6: Clinic Integration & Action Planning

- **Identify** local podiatry contacts.
- **Create** screening schedule & register.
- **Develop** simple Excel/Notebook tracker for monthly audits.
- **Pledge:** “10 patients screened in 10 days.”
- **Final assessment:** Knowledge quiz + OSCE.
- **Graduation & WhatsApp group initiation.**

5. ASSESSMENT BLUEPRINT

Component	Format	Weight	Pass Requirement
Knowledge Quiz	10 MCQs	20%	≥70%
Practical OSCE	4 stations (see Annex C)	60%	≥80% per station
Case Discussion	Group analysis, participation	20%	Active contribution
Overall Certification	Attendance ≥100%, all competencies	—	✓

Certification issued only to participants who pass all components.

6. COURSE EVALUATION & FOLLOW-UP

Participant Survey (Annex E):

- Confidence rating (1–10).
- Session usefulness (1–5).
- Open-ended feedback & improvement suggestions.

1-Month Follow-Up:

- Number of patients screened.
- Number referred.
- Barriers encountered.
- Success stories.

Quarterly Review (Facilitator):

- Screening coverage (% of diabetic patients).
- Referral timeliness.
- Amputation reduction trends in participating facilities.

7. FACILITATOR GUIDANCE

Before Course:

- Verify all equipment (monofilaments, models, handouts).
- Prepare translated patient materials.
- Arrange adequate lighting and hygiene setup.

During Course:

- Prioritize practice over lecture.
- Correct technique immediately.
- Encourage peer teaching and support.
- Adapt pace to group needs.

After Course:

- Add participants to WhatsApp support group.
- Conduct 2-week virtual check-in.
- Collect outcomes and photos for APP impact report.
- Schedule annual refresher invitations.

8. APPENDICES

Annex A – Screening Form

EDU-FOOT DIABETIC FOOT SCREENING FORM
(ASK–LOOK–FEEL–TEST–ACT layout with risk grid)

Annex B – WhatsApp Referral Template

Pre-formatted text for urgent and routine referrals.

Annex C – OSCE Rubric

4-station checklist with scoring grid.

Annex D – Competency Checklist

Monofilament / Pulse / Risk / Documentation skills.

Annex E – Participant Evaluation Form

Likert scale + open-ended feedback.

9. CERTIFICATION & CPD NOTES

- **Certificate:** EDU-FOOT Foot Screening Competency Certificate.
- **Validity:** 1 year.
- **Renewal:** 4-hour refresher course.
- **Advanced Pathways:** EDU-FOOT Advanced & Train-the-Trainer Modules.

CPD Credits (Pending SANC):

- Clinical Care: 8 CEUs

- Ethics & Human Rights: 1 CEU
- Education: 1 CEU

10. CONTACT & COPYRIGHT

EDU-FOOT Programme

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“When you save a foot, you save a life, a family, and a community’s hope.”

— Mazizi Njokweni

END OF DOCUMENT

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